Birth and Health Justice

Meghan Warner, PhD



Today

Birth Data

Ultrasounds

Continuous Fetal Heart Monitoring

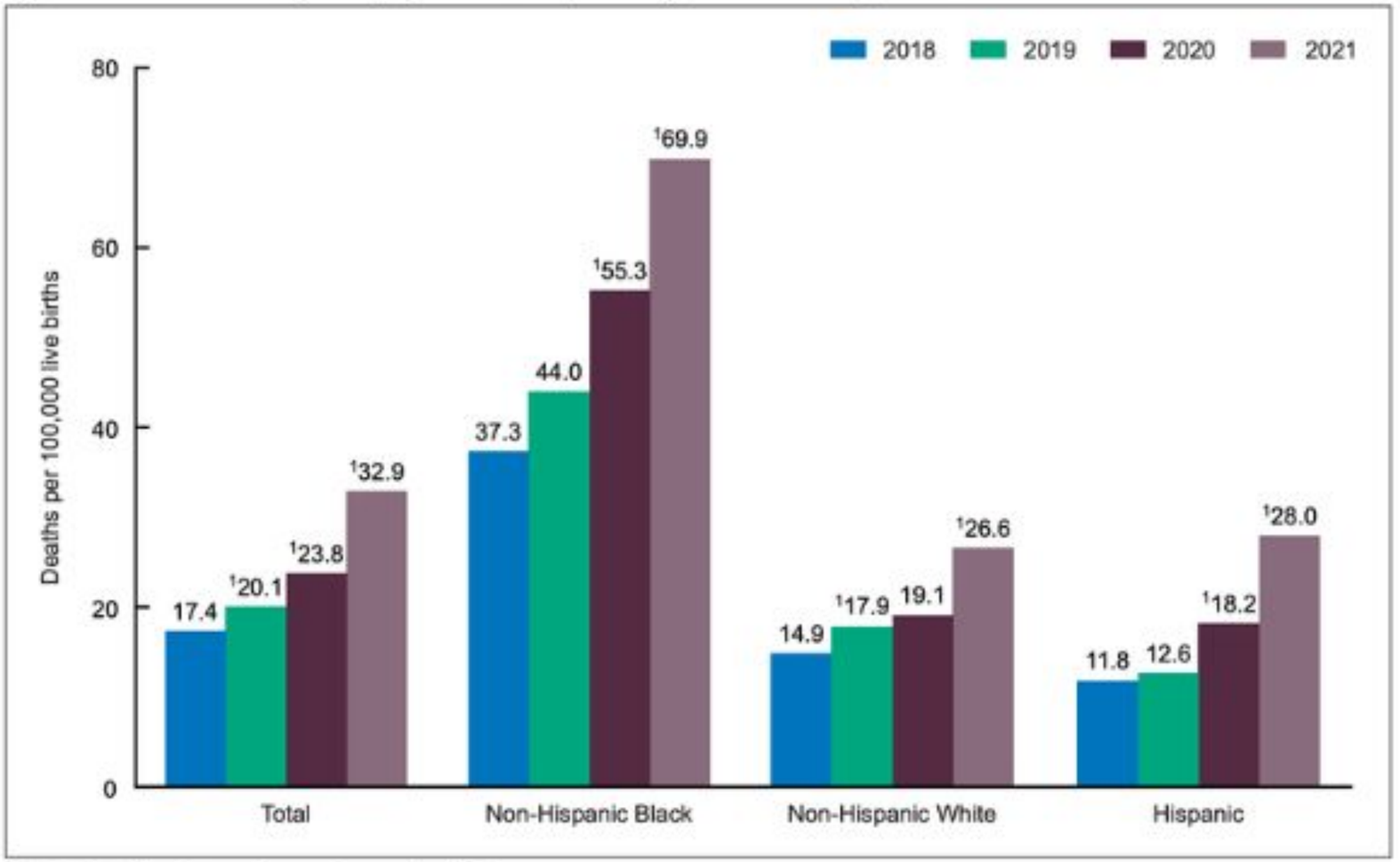
Questions?

What's the first word you think of when you hear "birth" (other than "birth")?

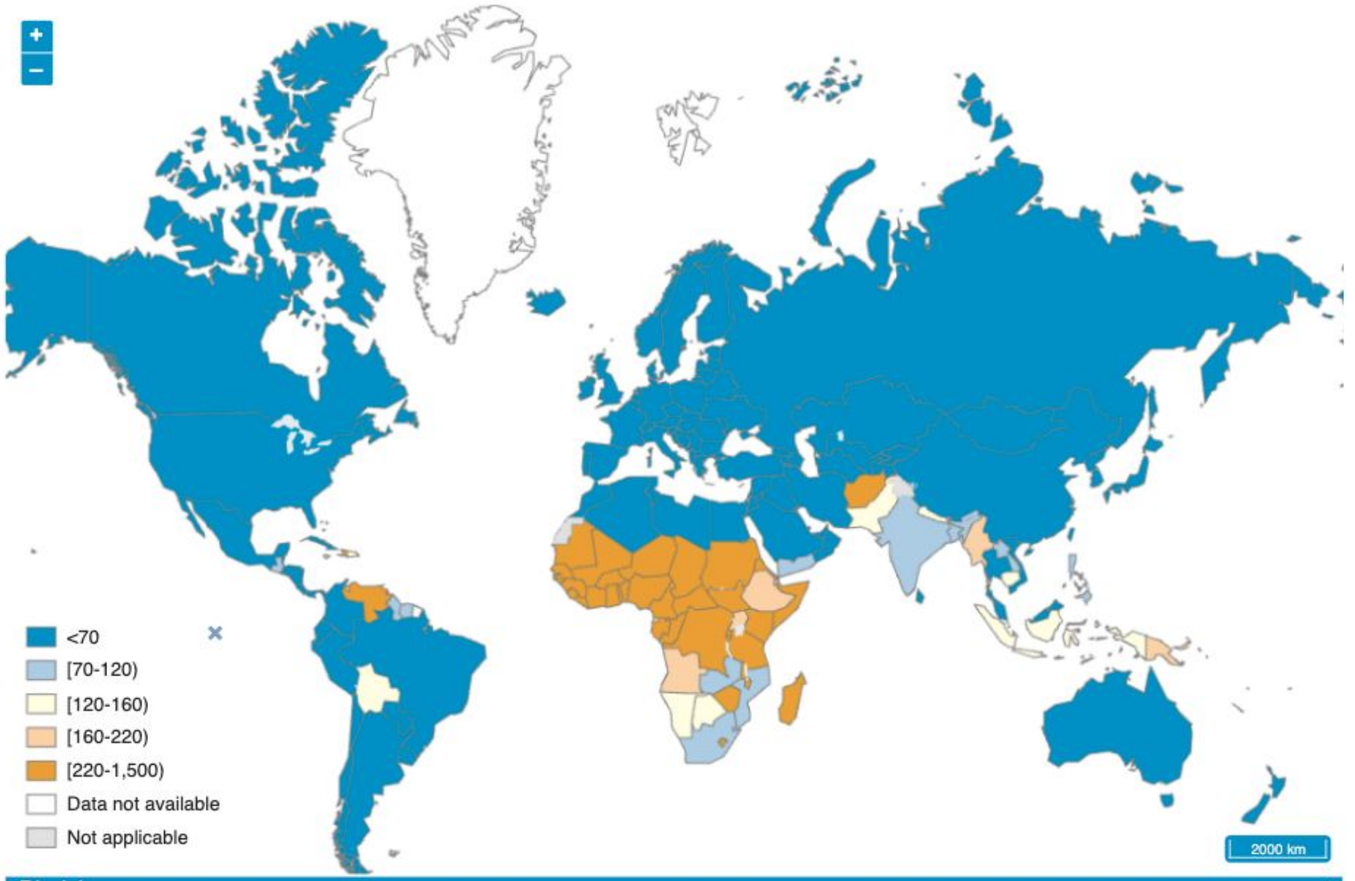


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Figure 1. Maternal mortality rates, by race and Hispanic origin: United States, 2018–2021



[1]Statistically significant increase from previous year ($p < 0.05$).
NOTE: Race groups are single race.
SOURCE: National Center for Health Statistics, National Vital Statistics System, Mortality.

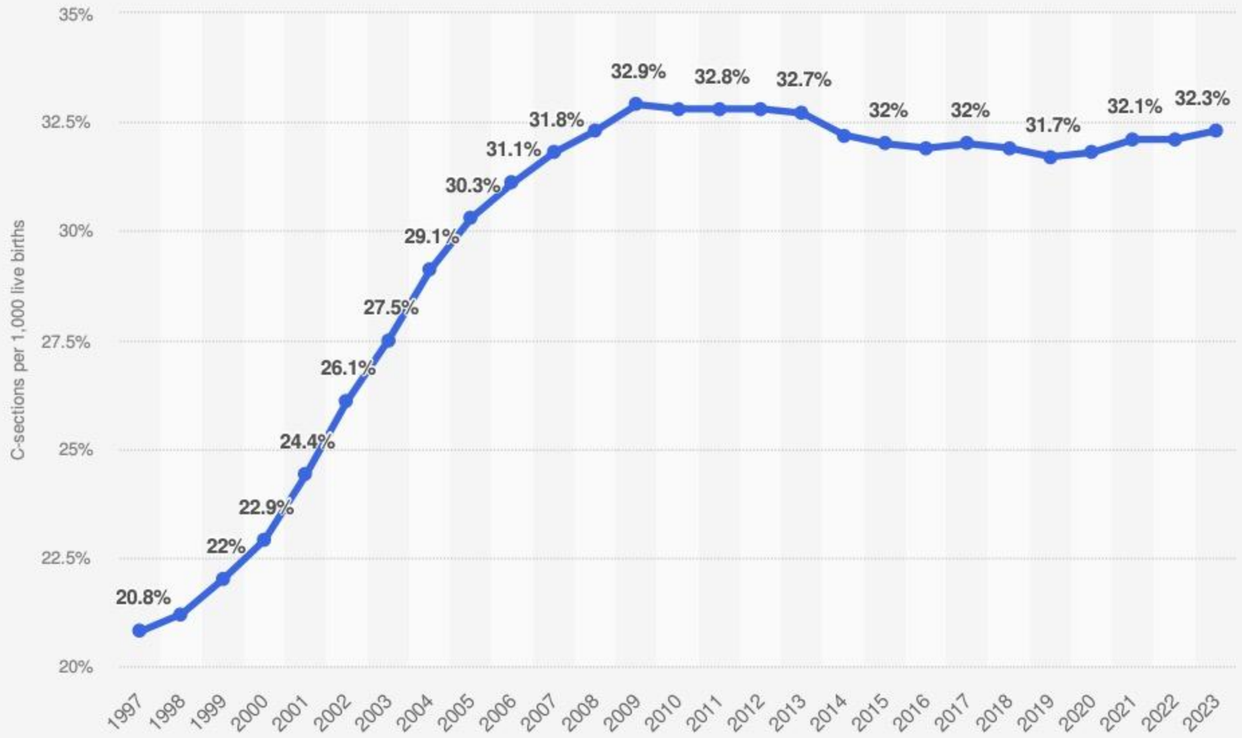


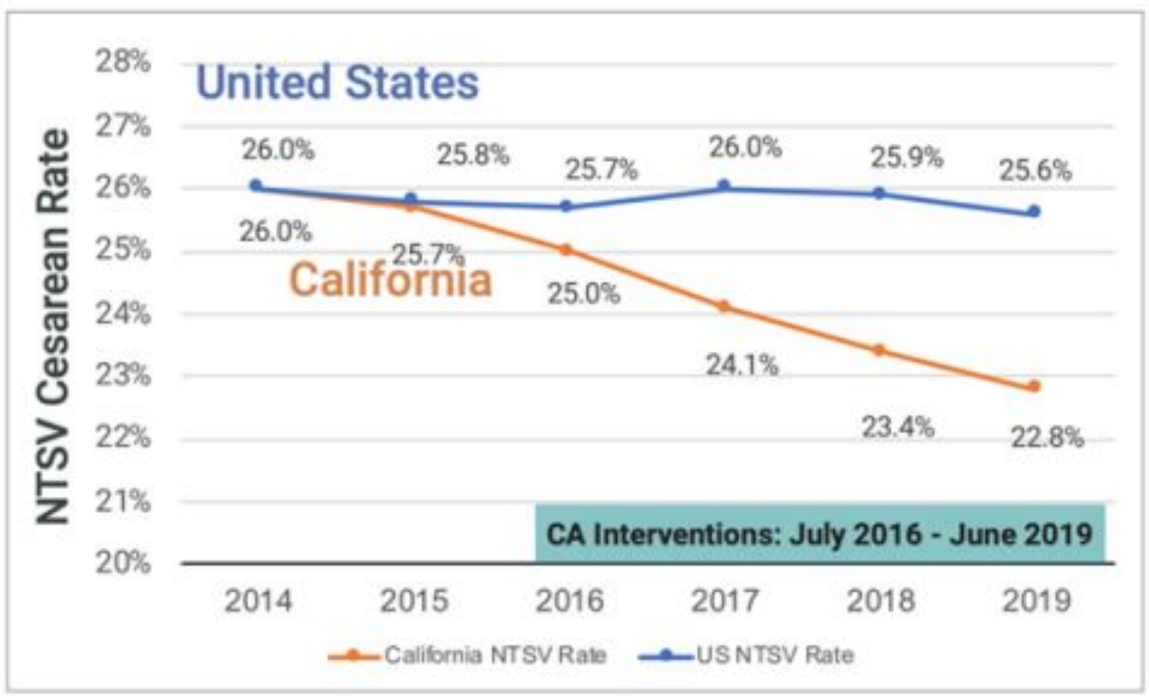
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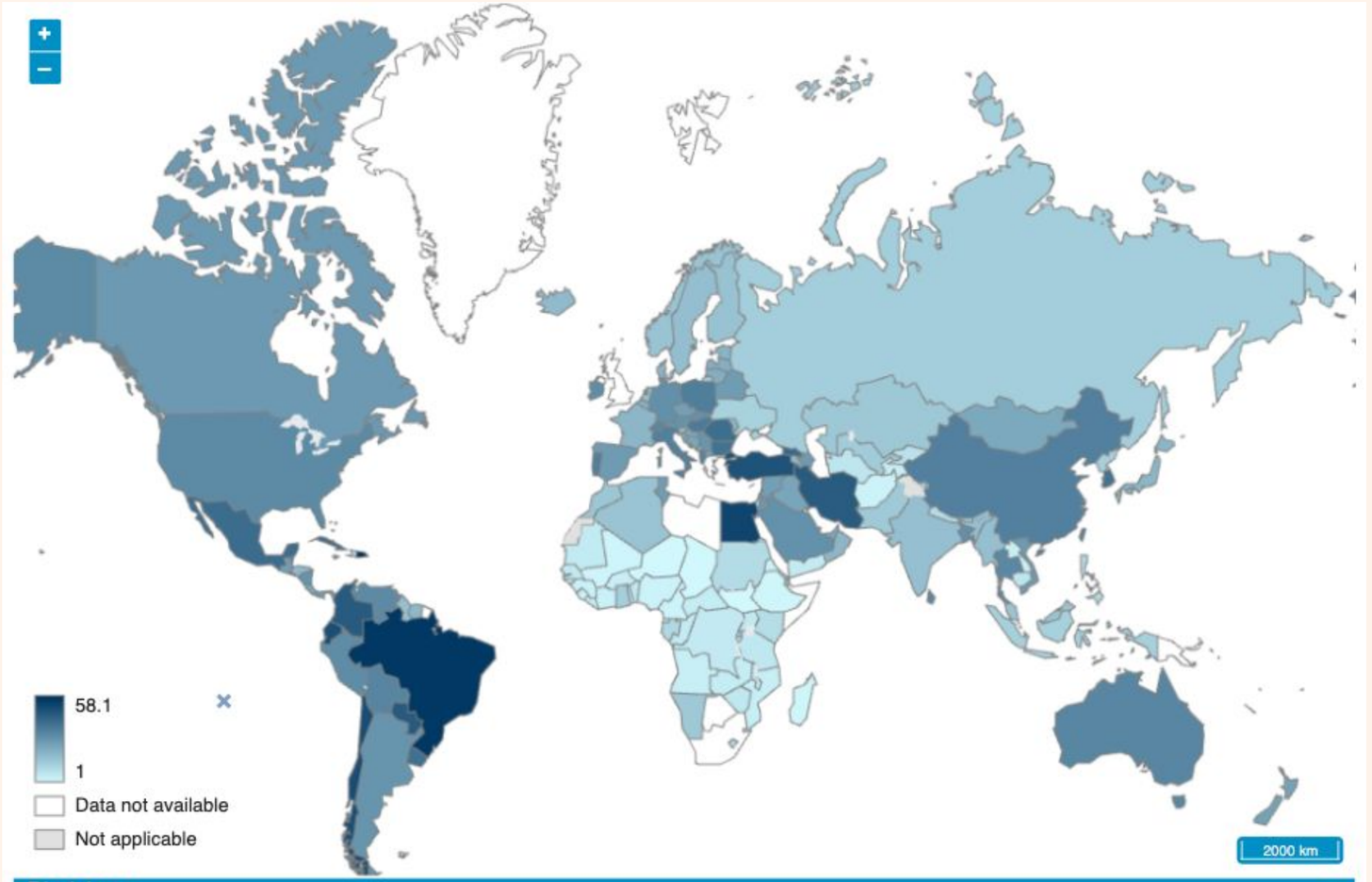
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Percentage of all live births by Cesarean delivery in the United States from 1997 to 2023
C-sections per 1,000 live births
35%
32.5%
30%
27.5%
25%
22.5%
20%
20.8%
22%
22.9%
24.4%
26.1%
27.5%
29.1%
30.3%
31.1%
31.8%
32.9%
32.8%
32.7%
32%
32%
31.7%
32.1%
32.3%
1997 1998 1999 2000 2001 2002 2003 2004 2005 2006 2007 2008 2009 2010 2011 2012 2013 2014 2015 2016 2017 2018 2019 2020 2021 2022 2023
Source
CDC
© Statista 2025
Additional Information:
United States; CDC; NCHS (NVSS); 1997 to 2023


NTSV Cesarean Rate
28%
27%
26%
25%
24%
23%
22%
21%
20%
United States
26.0%
25.8%
25.7%
26.0%
25.9%
25.6%
California
26.0%
25.7%
25.0%
24.1%
23.4%
22.8%
CA Interventions: July 2016 - June 2019
2014
2015
2016
2017
2018
2019
California NTSV Rate
US NTSV Rate



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Ultrasounds

- Benefits:
  - Better estimate due date
  - Determine location of fetus + placenta
  - Determine fetal number
  - Monitor fetal growth
  - Identify abnormalities
  - Identify breech babies

- Risks
  - Over/under diagnosis






With this powerful hand-held device, they can now offer
essential maternal health services

Continuous Fetal Heart Monitoring



- What: tool that measures fetal heart rate and contraction strength
- Introduced in 1960s and 1970s *without evidence*
- Theory: constantly monitoring fetus will allow us to intervene when necessary, reducing complications due to hypoxia (low oxygen)
- Benefit: providers can remotely (or semi-remotely) monitor more laboring people



Intermittent Auscultation (IA)





- Cochrane review:
  - CFM *not* associated with less *perinatal death* or *lower rates of cerebral palsy* compared with IA
  - CFM *was* associated with *higher cesarean* rates and higher instrumental vaginal deliveries compared with IA

- Why?
  - Subjective interpretations
  - Cascade of interventions

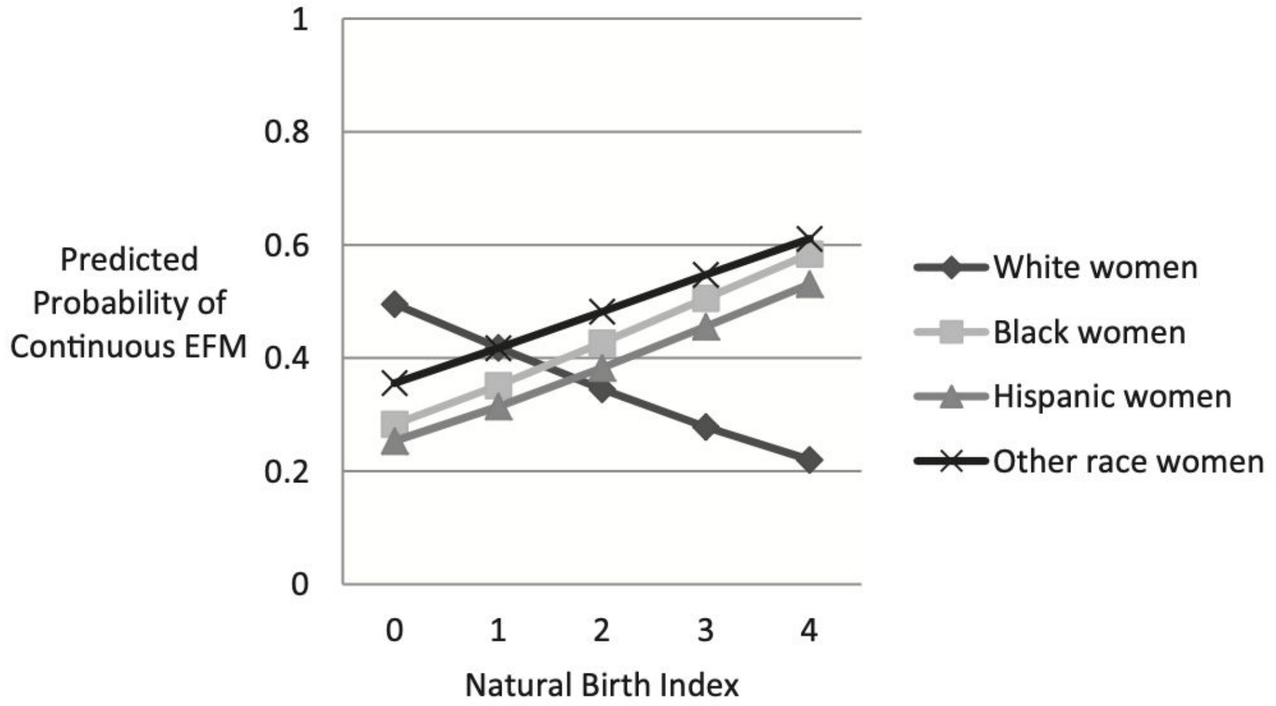
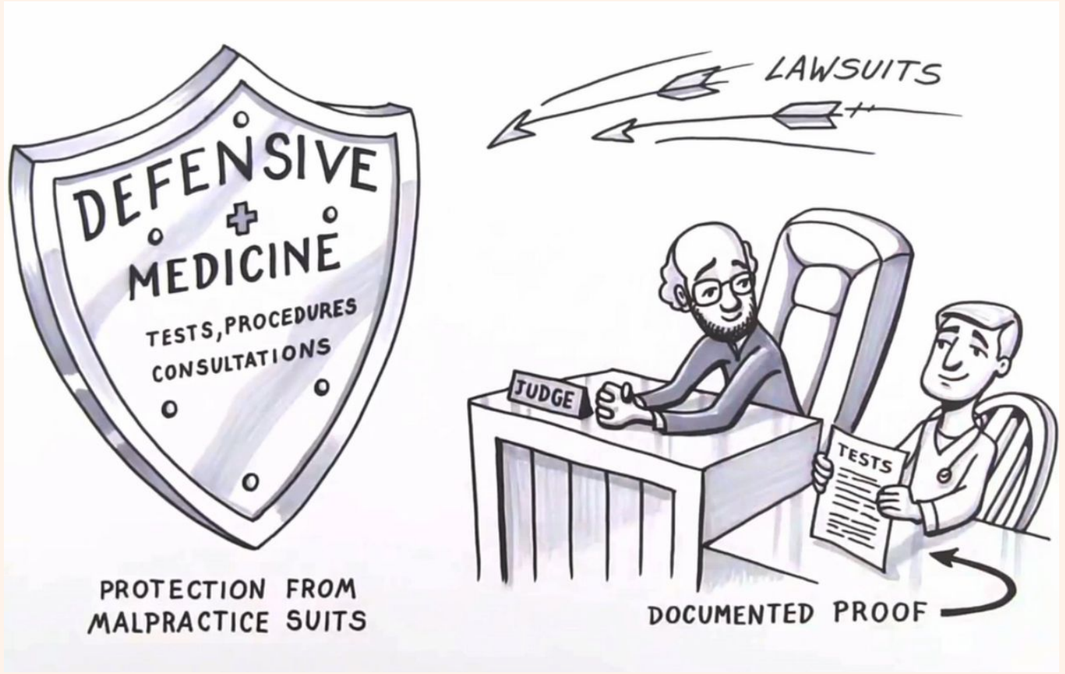


Figure 1. Predicted Probability of Interventions (Epidural, Continuous EFM) by Race x Natural Birth Index

Johnson, Katherine M., and Richard M. Simon. 2021. "Privilege in the Delivery Room? Race, Class, and the Realization of Natural Birth Preferences, 2002–2013." *Social Problems* 68(3):552–73.

Why Continued CFM?



“Michelle” - OB

“It's great surgery, you pull out the baby, it cries, and everybody's happy, and everybody says, ‘thank you.’
And then the patient's like ‘my doctor did a C-Section because the baby was struggling.’ And I'm like, [dubious] ‘okay.’
And then there was nuchal cord, and you're like, ‘AHA!, I saved your baby. There was a cord wrapped around the neck.’
There's all these ways that we can create a narrative that that was what was necessary, which again is helpful to the patient, because she doesn't want to know that it wasn't totally necessary. So, it's a very tricky outcome.”

Faden & Powers + Health Justice



- Does a piece of biotechnology help us achieve a *sufficient level of health*?
- How does a piece of biotechnology relate to *patterns of oppression*?
- How do we know?
  - What evidence do we have?
  - Who conducted the research, and who was studied?
  - What else do we need to know?

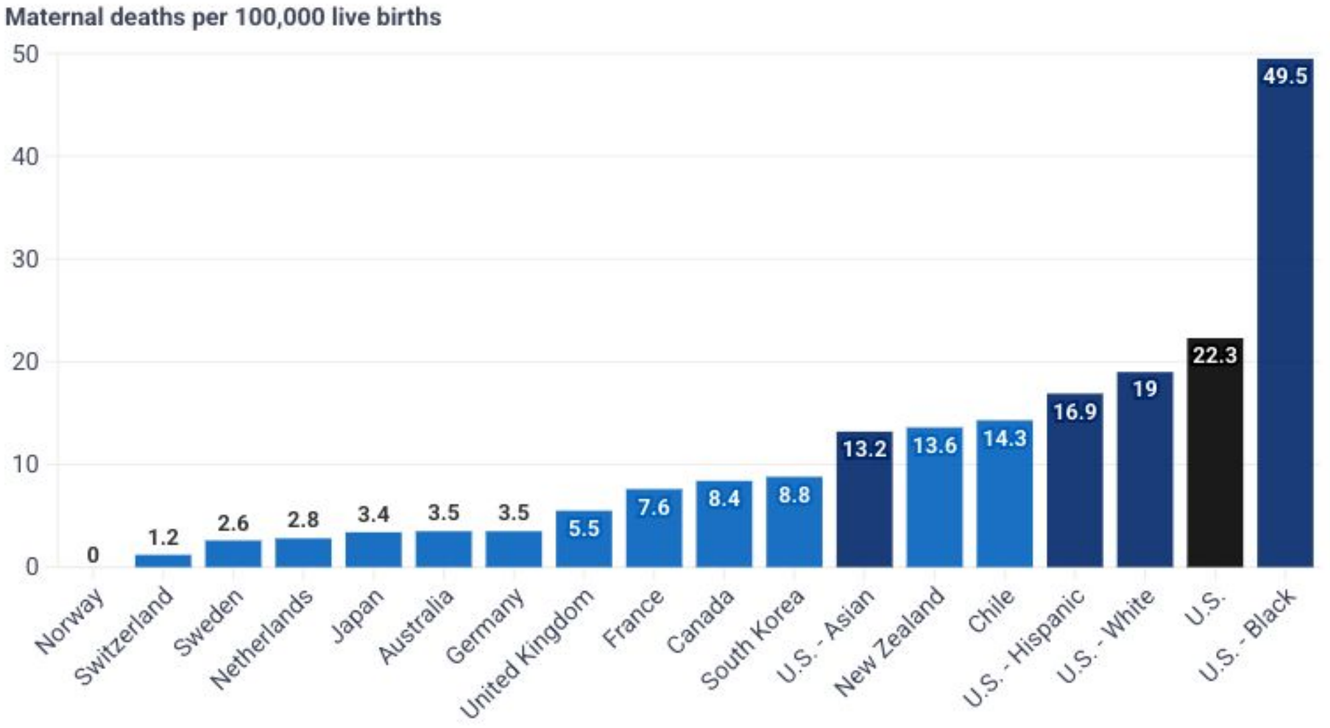
Pose a question. This could be a discussion question for section (based on lecture or readings) or a question for Dr. Warner.

Nobody has responded yet.

Hang tight! Responses are coming in.

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U.S. Maternal Mortality vs. Other High-Income Countries

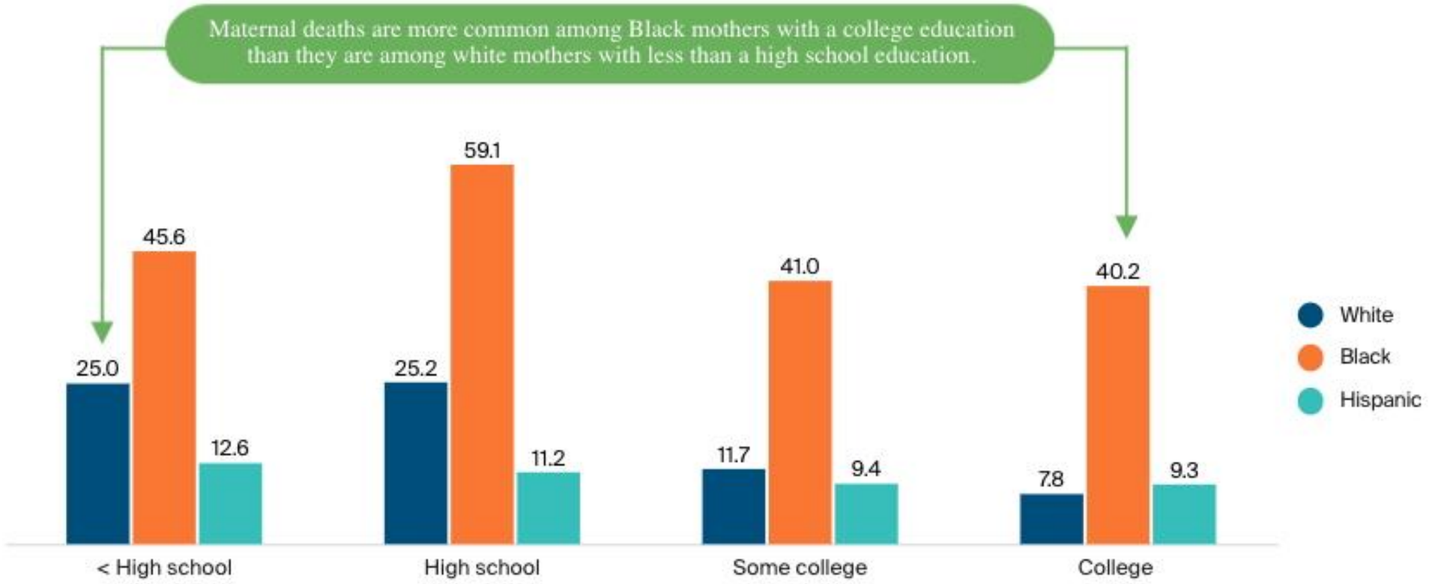


Source: The Commonwealth Fund • Chart: Julia Haines/U.S. News & World Report

All country data from OECD Health Statistics 2023 extracted on February 29, 2024, except data for US are 2022 data from the Centers for Disease Control and Prevention. 2015 data for FRA; 2017 data for UK; 2018 data for NZ; 2020 data for CAN and SWIZ; 2021 data for AUS, GER, JPN, KOR, NETH and SWE; 2022 data for CHL (provisional), NOR and US. The maternal mortality ratio is defined as the death of a woman while pregnant or within 42 days of termination of pregnancy, irrespective of the duration and site of the pregnancy, from any cause related to or aggravated by the pregnancy or its management but not from accidental or incidental causes.

Even higher education does not protect Black mothers from pregnancy-related death.

Pregnancy-related mortality ratios per 100,000 births in the U.S., 2007–2016

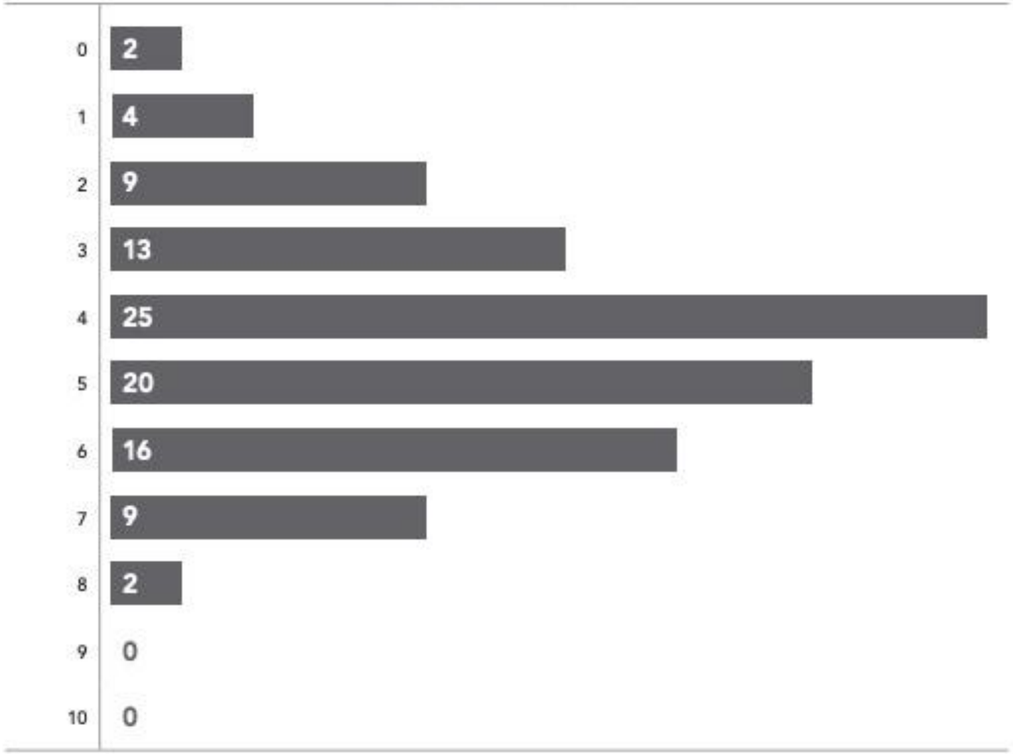


Download data

Data: Emily E. Petersen et al., "Racial/Ethnic Disparities in Pregnancy-Related Deaths — United States, 2007–2016," *Morbidity and Mortality Weekly Report* 68, no. 35 (Sept. 6, 2019): 762–65.

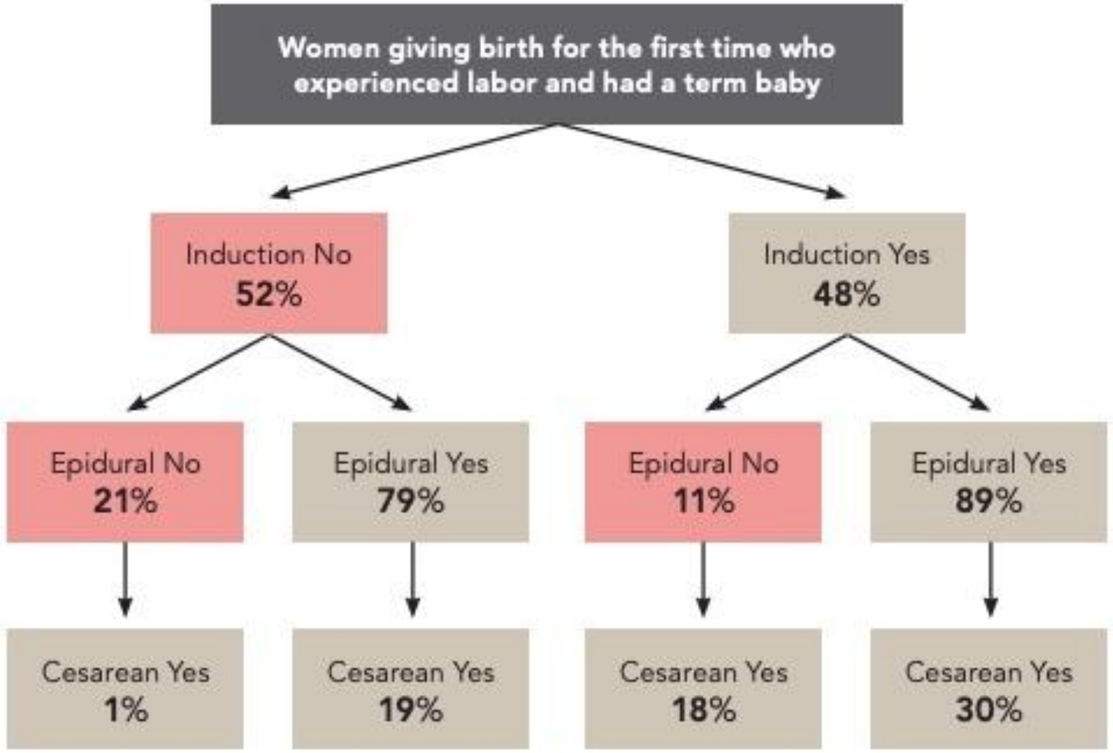
Source: Eugene Declercq and Laurie Zephyrin, *Maternal Mortality in the United States: A Primer* (Commonwealth Fund, Dec. 2020). https://doi.org/10.26099/ta1q-mw24

Figure 27: Cumulative Number of Selected Interventions Experienced Around the Time of Birth



Base: All women (n=2536)
Note: Interventions include sweeping membranes, artificial rupture of membranes, synthetic oxytocin to induce and/or speed up labor, bladder catheter, intravenous line, electronic fetal monitoring, epidural for pain, narcotics for pain, vacuum or forceps, and cesarean birth.
Note: Vacuum or forceps data obtained from respondents' birth certificates.

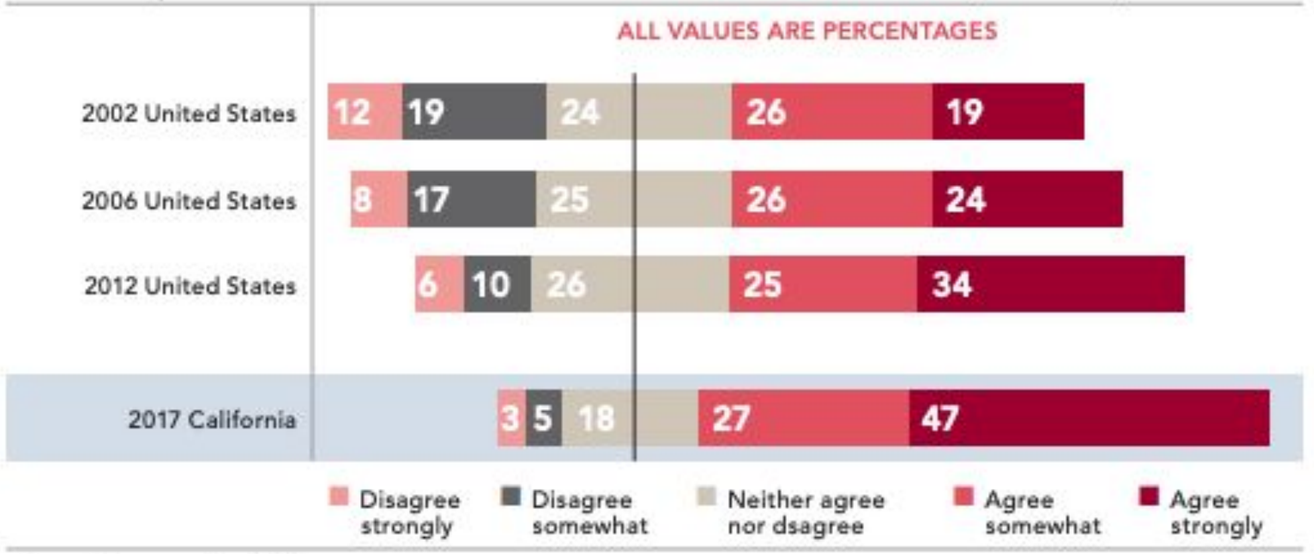
Figure 28: Cascade of Intervention



Base: Women giving birth for the first time, who experienced labor and had a term baby (n=841)
$p < .01$ for difference in cesarean rate by whether had induction and epidural
Note: In this group, which included 80% of women giving birth for the first time, the overall epidural rate was 84% and overall cesarean rate was 22%.

Figure 12: Childbirth Beliefs, United States, 2002–2012, and California, 2017

"How much do you agree or disagree with the following statement? Childbirth is a process that should not be interfered with unless medically necessary."



Base: All women (n=2451)